

2025 Audited Financial Statements

December 31, 2025



Sutter Health and Affiliates

Consolidated Financial Statements

Years ended December 31, 2025 and 2024

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Report of Independent Auditors

Sutter Health Board of Directors
Sutter Health and Affiliates

Opinion

We have audited the consolidated financial statements of Sutter Health and Affiliates (Sutter), which comprise the consolidated balance sheets as of December 31, 2025 and 2024, and the related consolidated statements of operations and changes in net assets and cash flows for the years then ended, and the related notes (collectively referred to as the “financial statements”).

In our opinion, the accompanying financial statements present fairly, in all material respects, the financial position of Sutter at December 31, 2025 and 2024, and the results of its operations, changes in its net assets and its cash flows for the years then ended in accordance with accounting principles generally accepted in the United States of America.

Basis for Opinion

We conducted our audits in accordance with auditing standards generally accepted in the United States of America (GAAS). Our responsibilities under those standards are further described in the Auditor’s Responsibilities for the Audit of the Financial Statements section of our report. We are required to be independent of Sutter and to meet our other ethical responsibilities in accordance with the relevant ethical requirements relating to our audits. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our audit opinion.

Responsibilities of Management for the Financial Statements

Management is responsible for the preparation and fair presentation of the financial statements in accordance with accounting principles generally accepted in the United States of America, and for the design, implementation, and maintenance of internal control relevant to the preparation and fair presentation of financial statements that are free of material misstatement, whether due to fraud or error.

In preparing the financial statements, management is required to evaluate whether there are conditions or events, considered in the aggregate, that raise substantial doubt about Sutter’s ability to continue as a going concern for one year after the date that the financial statements are issued.



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Auditor's Responsibilities for the Audit of the Financial Statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free of material misstatement, whether due to fraud or error, and to issue an auditor's report that includes our opinion. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS will always detect a material misstatement when it exists. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Misstatements are considered material if there is a substantial likelihood that, individually or in the aggregate, they would influence the judgment made by a reasonable user based on the financial statements.

In performing an audit in accordance with GAAS, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the amounts and disclosures in the financial statements.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of Sutter's internal control. Accordingly, no such opinion is expressed.
- Evaluate the appropriateness of accounting policies used and the reasonableness of significant accounting estimates made by management, as well as evaluate the overall presentation of the financial statements.
- Conclude whether, in our judgment, there are conditions or events, considered in the aggregate, that raise substantial doubt about Sutter's ability to continue as a going concern for a reasonable period of time.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit, significant audit findings, and certain internal control-related matters that we identified during the audit.

Ernst & Young LLP

March 4, 2026

Sutter Health and Affiliates

Consolidated Balance Sheets

(in millions)

	December 31,	
	2025	2024
Assets		
Current assets:		
Cash and cash equivalents	\$ 879	\$ 875
Short-term investments	10,154	8,380
Patient accounts receivable	1,867	1,792
Other	1,474	1,170
Total current assets	14,374	12,217
Non-current assets:		
Non-current investments	2,199	2,072
Property, plant and equipment, net	8,804	7,559
Other	1,615	1,817
Total non-current assets	12,618	11,448
Total assets	\$ 26,992	\$ 23,665
Liabilities and net assets		
Current liabilities:		
Short-term borrowings	\$ 199	\$ –
Accounts payable	929	848
Accrued salaries and employee benefits	1,010	920
Other accrued expenses	1,244	1,406
Current portion of long-term obligations	43	347
Total current liabilities	3,425	3,521
Non-current liabilities:		
Long-term obligations, less current portion	6,354	4,858
Other	1,372	1,509
Total non-current liabilities	7,726	6,367
Total liabilities	11,151	9,888
Net assets:		
Without donor restrictions:		
Controlling interests	15,061	13,021
Noncontrolling interests	76	96
With donor restrictions	704	660
Total net assets	15,841	13,777
Total liabilities and net assets	\$ 26,992	\$ 23,665

See accompanying notes to consolidated financial statements.

Sutter Health and Affiliates

Consolidated Statements of Operations and Changes in Net Assets

(in millions)

	Year ended December 31,	
	2025	2024
Net assets without donor restrictions:		
Revenues:		
Patient service revenues	\$ 17,078	\$ 15,812
Premium revenues	2,208	2,009
Contributions	160	55
Other	374	331
Total revenues	19,820	18,207
Expenses:		
Salaries and employee benefits	8,841	8,058
Purchased services	5,642	5,125
Supplies	2,688	2,346
Depreciation and amortization	755	702
Interest	213	178
Other	1,172	1,656
Total expenses	19,311	18,065
Income from operations	509	142
Nonoperating income:		
Investment income, net	1,364	771
Other components of net periodic postretirement cost	59	116
Other	(73)	(21)
Total nonoperating income, net	1,350	866
Net income	1,859	1,008
Less income attributable to noncontrolling interests	(98)	(99)
Income attributable to controlling interests	1,761	909

Sutter Health and Affiliates

Consolidated Statements of Operations and Changes in Net Assets (continued)

(in millions)

	Year ended December 31,	
	2025	2024
Net assets without donor restrictions (continued):		
Controlling interests:		
Income attributable to controlling interests	\$ 1,761	\$ 909
Change in net unrealized gains and losses on investments classified as other-than-trading	3	5
Net assets released from restriction for equipment acquisition	41	24
Postretirement-related changes other than net periodic postretirement cost	240	289
Other	(5)	59
Increase in controlling interests	2,040	1,286
Noncontrolling interests:		
Income attributable to noncontrolling interests	98	99
Distributions	(90)	(94)
Other	(28)	(1)
(Decrease) increase in noncontrolling interests	(20)	4
Net assets with donor restrictions:		
Contributions	81	136
Investment income	21	18
Change in net unrealized gains and losses on investments	15	7
Net assets released from restriction	(71)	(49)
Other	(2)	—
Increase in net assets with donor restrictions	44	112
Increase in net assets	2,064	1,402
Net assets at beginning of year	13,777	12,375
Net assets at end of year	\$ 15,841	\$ 13,777

See accompanying notes to consolidated financial statements.

Sutter Health and Affiliates

Consolidated Statements of Cash Flows

(in millions)

	Year ended December 31,	
	2025	2024
Operating activities		
Increase in net assets	\$ 2,064	\$ 1,402
Adjustments to reconcile increase in net assets to net cash provided by operating activities:		
Depreciation and amortization	739	694
Amortization of bond issuance costs, premium and discount, net	(21)	(22)
Net realized gains and losses and change in net unrealized gains and losses on investments	(1,160)	(569)
Contributions of buildings and other net assets	–	(112)
Restricted contributions and related investment income	(102)	(103)
Distributions to noncontrolling interests	90	94
Change in net postretirement benefits	(59)	(163)
Net changes in operating assets and liabilities:		
Patient accounts receivable	(75)	(78)
Other assets	71	(293)
Accounts payable and accrued expenses	42	325
Other non-current liabilities	(137)	282
Net cash provided by operating activities	1,452	1,457
Investing activities		
Purchases of property, plant and equipment	(2,030)	(819)
Payments for business acquisitions	(20)	(32)
Proceeds from disposal of property, plant and equipment	3	8
Purchases of investments	(8,015)	(9,330)
Proceeds from sales of investments	7,274	9,085
Other	(85)	(35)
Net cash used in investing activities	(2,873)	(1,123)

Sutter Health and Affiliates

Consolidated Statements of Cash Flows (continued)

(in millions)

	Year ended December 31,	
	2025	2024
Financing activities		
Payment on commercial paper	\$ (300)	\$ —
Proceeds from borrowings on commercial paper	500	—
Payments of long-term obligations	(547)	(50)
Proceeds from issuance of long-term obligations	1,767	23
Bond issuance costs	(7)	—
Restricted contributions and related investment income	102	103
Distributions to noncontrolling interests	(90)	(94)
Net cash provided by (used in) financing activities	1,425	(18)
Net increase in cash and cash equivalents	4	316
Cash and cash equivalents at beginning of year	875	559
Cash and cash equivalents at end of year	\$ 879	\$ 875

See accompanying notes to consolidated financial statements.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements

December 31, 2025 and 2024

(in millions)

1. ORGANIZATION

Sutter Health is a California not-for-profit corporation that is the parent of a multi-provider integrated health care delivery system headquartered in Northern California, which includes various health care-related businesses operating primarily in Northern California. Sutter Health and its affiliates and subsidiaries provide health care, education, research and administrative services.

Sutter Health's integrated health care delivery system includes acute care, medical foundations, fundraising foundations, and a variety of other specialized health care services. These entities are commonly referred to as the affiliates. Most acute care hospitals provide a full range of medical services (e.g., surgical, intensive care, emergency room, and obstetrics). All emergency rooms provide emergency care, regardless of a patient's ability to pay. Sutter Health and its affiliates also serve their communities with various programs, such as health education, health libraries, school-based clinics, home health care, hospice care, adult day care, prenatal clinics, community clinics, immunization services, and health professions education.

2. ACCOUNTING POLICIES

Basis of Consolidation: The Sutter Health and Affiliates consolidated financial statements include the accounts of Sutter Health and its controlled affiliates and subsidiaries (Sutter). All significant intercompany accounts and transactions have been eliminated in consolidation.

Use of Estimates: The preparation of financial statements in conformity with accounting principles generally accepted in the United States (U.S.) requires management to make estimates, judgments and assumptions that affect the amounts reported and disclosed in the consolidated financial statements. Although estimates are considered to be fairly stated at the time made, actual results could differ materially from those estimates.

Cash Equivalents: Cash equivalents include all highly liquid investments with original maturities of 90 days or less, including money market accounts with limited market risk, and investment-grade debt instruments, many of which are backed by the U.S. Government or other government agencies. Financial instruments that potentially subject Sutter to concentrations of credit risk include cash equivalents and investments. Cash equivalents are stated at fair market value.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

2. ACCOUNTING POLICIES (continued)

Investments: Investments consist principally of U.S. and foreign equity, corporate and government securities, a hedge fund portfolio, and private equity funds, all of which are carried at fair value or net asset value (NAV) as a practical expedient to estimate fair value. These investments include financial instruments, which are valued using the most current information available, which may be less current than the date of these consolidated financial statements. Short-term investments consist of investments with an original maturity of more than three months and could be utilized within one year. Non-current investments consist of investments that cannot be utilized within one year, including certain investments held in trust, and investments designated by the appropriate Sutter governing boards for future capital improvements.

Derivative Instruments: Sutter offsets fair value amounts recognized for certain derivative transactions from contracts executed with the same counterparty under a master netting arrangement. As a result, there is no net exposure to counterparties at December 31, 2025 and 2024.

Securities Lending: Sutter participates in securities lending transactions with its investment custodian, whereby Sutter lends a portion of its securities to various brokers in return for securities as collateral for the securities loaned, usually on a short-term basis. Noncash collateral provided by the brokers generally approximates 102% to 105% of the fair value of the securities on loan and is adjusted for daily market fluctuations. Sutter earns a rebate on the loaned securities. Neither Sutter nor its investment custodian has the ability to pledge or sell securities received as collateral unless a borrower defaults; therefore, these transactions are not recorded on the Consolidated Balance Sheets. As of December 31, 2025 and 2024, the fair value of securities on loan is \$553 and \$370, respectively, and the related noncash collateral is \$601 and \$396, respectively. The majority of the securities on loan are U.S. equity securities and U.S. government securities and are collateralized by U.S. and Foreign equity securities.

Sutter's noncontributory defined benefit plan also participates in a securities lending arrangement (see Note 12).

Patient Accounts Receivable: Sutter's primary concentration of credit risk is patient accounts receivable, which consist of amounts owed from patients and third-party payers. Sutter manages the receivables by regularly reviewing its patient accounts and contracts.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

2. ACCOUNTING POLICIES (continued)

Significant concentrations of Patient accounts receivable are as follows:

	December 31,	
	2025	2024
Medicare	\$ 477	\$ 468
Medi-Cal	220	242
Commercial	1,130	1,025
Other	40	57
	<u>\$ 1,867</u>	<u>\$ 1,792</u>

Inventories: Inventories, which consist principally of medical and other supplies, are stated on the basis of cost determined by the first-in, first-out method, which is not in excess of market.

Property, Plant and Equipment: Property, plant and equipment are stated on the basis of cost or, in the case of donated items, on the basis of fair market value at the date of donation, less depreciation and any impairment write-downs. Equipment includes medical equipment, furniture and fixtures, software, and internally developed software. Routine maintenance and repairs are charged to expense as incurred. Expenditures that increase values, change capacities, or extend useful lives are capitalized, as is interest on amounts borrowed to finance constructed assets during the construction phase. Sutter accrued obligations for property, plant and equipment of \$123 and \$89 as of December 31, 2025 and 2024, respectively.

Depreciation is computed by the straight-line method over the estimated useful lives of the assets, which generally range from 2 to 70 years for buildings and improvements and from 2 to 20 years for equipment. Leasehold improvements are amortized using the straight-line method over the shorter of the useful life or lease term, which range from 2 to 40 years. Amortization of equipment under finance leases is included in depreciation and amortization expense.

Asset Impairment: Sutter considers whether indicators of impairment are present, and if indicators are present, performs the necessary steps to determine if the carrying value of an asset is recoverable. There were no material impairment charges recorded for the years ended December 31, 2025 and 2024.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

2. ACCOUNTING POLICIES (continued)

Other Assets: Goodwill represents the excess of purchase price over the fair value of net assets acquired. Goodwill and other intangible assets acquired in business combinations that have indefinite useful lives are subject to impairment tests. Sutter performs impairment tests at the reporting unit level annually or when events occur that require an evaluation to be performed. If the carrying value of goodwill is determined to be impaired, or if the carrying value of a business that is to be sold or otherwise disposed of exceeds its fair value, the carrying value is reduced, including any allocated goodwill, to fair value. Estimates of fair value are based on appraisals, established market prices for comparative assets, or internal estimates of future net cash flows based on projected performance, depending on circumstances.

The changes in the carrying amount of goodwill, which are included in Other non-current assets, are as follows:

	December 31,	
	2025	2024
Goodwill at beginning of year	\$ 153	\$ 125
Additions	20	28
Goodwill at end of year	<u>\$ 173</u>	<u>\$ 153</u>

Liquidity Management: As part of its liquidity management, Sutter's strategy is to structure its financial assets to be available to satisfy general expenses, current liabilities, and other obligations as they come due. Sutter invests cash in excess of daily requirements in investments and has a committed syndicated line of credit and a commercial paper program, as discussed in Note 7, to help manage unanticipated liquidity needs.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

2. ACCOUNTING POLICIES (continued)

Sutter's financial assets available for general expenses within one year are as follows:

	December 31,	
	2025	2024
Cash and cash equivalents	\$ 879	\$ 875
Short-term investments	10,154	8,380
Patient accounts receivable	1,867	1,792
Other current assets	1,032	781
	<u>\$ 13,932</u>	<u>\$ 11,828</u>

Other Liabilities: Other non-current liabilities consist of: (i) insurance liabilities, including estimated liabilities for professional liability and comprehensive general liability losses, and workers' compensation, (ii) the portion of estimated third-party settlements not expected to be settled within a year, (iii) hospital fee program liabilities, (iv) operating lease liabilities, and (v) certain other liabilities.

General and Professional Liability and Other Liability Risks: Sutter's insurance programs combine various levels of self-insurance with reinsurance, excess commercial insurance coverage, and stop-loss policy agreements with independent insurance companies to limit its losses on professional liability, comprehensive general liability, workers' compensation, and employee health claims. The provisions and reserves for estimated professional liability and comprehensive general liability claims, workers' compensation, and employee health include estimates of the ultimate costs for both uninsured reported claims and claims incurred-but-not-reported, in accordance with actuarial projections or paid claims lag models based on historical experience. Professional liabilities and comprehensive general liabilities were \$163 and \$148, discounted at a rate of 3.8% and 4.6%, as of December 31, 2025 and 2024, respectively. Workers' compensation liabilities were \$216 and \$224, discounted at a rate of 4.2% and 4.8%, as of December 31, 2025 and 2024, respectively. Employee health liabilities were \$146 and \$117 as of December 31, 2025 and 2024, respectively, and were recorded on an undiscounted basis. These estimates are subject to a significant degree of inherent variability. Estimates are continually monitored and reviewed, and as reserves are adjusted the differences are reflected in current operations.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

2. ACCOUNTING POLICIES (continued)

In lieu of a workers' compensation security deposit requirement, Sutter paid assessment charges to participate in the California Self-Insurers' Alternative Security Program, which provided coverage of \$263 and \$280 as of December 31, 2025 and 2024, respectively.

Contingencies: Estimated losses from contingencies are recorded when they are probable and reasonably estimable.

Net Assets: Net resources that are not restricted by donors are included in Net assets without donor restrictions. Gifts of long-lived operating assets, such as property, plant or equipment, are reported as Net assets without donor restrictions and excluded from income. Resources restricted by donors for a specified time or purpose are reported as Net assets with donor restrictions.

When the specific purposes are met, either through passage of a stipulated time period or when the purpose for restriction is accomplished, they are released to Other revenues in the Consolidated Statements of Operations and Changes in Net Assets. Resources restricted by donors for additions to property, plant and equipment are initially reported as Net assets with donor restrictions and are transferred to Net assets without donor restrictions when expended. Donor-imposed restrictions, which stipulate that the resources be maintained permanently, are reported as Net assets with donor restrictions.

Investment income related to net assets with donor restrictions is classified as either Net assets without donor restrictions or Net assets with donor restrictions based on the intent of the donor.

Purchased Services: Purchased services expense consists of a variety of contracted and other purchased services, including medical group compensation, other professional fees, repairs and maintenance, capitated purchased services, and revenue cycle management services. Medical group compensation is recorded by Sutter in accordance with professional services agreements between affiliated medical foundations and contracted medical groups. Capitated purchased services include paid claims, sub-capitation payments, stop-loss payments, and accruals for incurred-but-not-reported estimates based on historical experience and available data.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

2. ACCOUNTING POLICIES (continued)

Research and Development: Research and development costs are expensed as incurred. Research and development expense, included in Total expenses, was \$83 and \$74 for the years ended December 31, 2025 and 2024, respectively.

Income Taxes: Sutter Health and certain affiliates have been determined to be exempt organizations by the Internal Revenue Service and the California Franchise Tax Board and generally are not subject to taxes on income. Certain activities of Sutter are subject to income taxes; however, such activities are not significant to the consolidated financial statements. With respect to its taxable activities, Sutter records income taxes using the liability method, under which deferred tax assets and liabilities are determined based on the differences between the financial accounting and tax basis of assets and liabilities. Deferred tax assets or liabilities at the end of each period are determined using the currently enacted tax rate expected to apply to taxable income in the periods that the deferred tax asset or liability is expected to be realized or settled.

Sutter recognizes the tax benefit from uncertain tax positions, when it is more likely than not that the tax positions will be sustained upon examination by the tax authorities, based on the technical merits of the position. The tax benefit is measured based on the largest benefit that has a greater than 50% likelihood of being realized upon ultimate settlement. The statute of limitations for tax years 2022 through 2024 remains open in U.S. tax jurisdictions in which Sutter and its affiliates are subject to taxation. There were no such uncertain tax positions recognized for the years ended December 31, 2025 and 2024.

Investment income, net: Investment income, net includes dividend and interest income reported net of investment fees, which are recorded on an accrual basis. It also includes net realized gains and losses and change in net unrealized gains and losses on trading securities. Purchases and sales of trading securities are recorded on the trade date basis. Dividends are recorded on the ex-dividend dates.

Performance Indicator: Net income and Income attributable to controlling interests, as reflected in the Consolidated Statements of Operations and Changes in Net Assets, are performance indicators. The performance indicators include all changes in Net assets without donor restrictions, excluding Net assets released from restriction for equipment acquisition, Change in net unrealized gains and losses on investments classified as other-than-trading, Postretirement-related changes other than net periodic postretirement cost, and Other changes.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

3. INVESTMENTS AND INVESTMENT INCOME

Investments are held for the following uses:

	December 31,	
	2025	2024
Board-designated	\$ 401	\$ 354
Investments	11,952	10,098
	12,353	10,452
Less short-term investments	(10,154)	(8,380)
Non-current investments	\$ 2,199	\$ 2,072

Investment income, net consists of the following:

	Year ended December 31,	
	2025	2024
Fixed income securities	\$ 109	\$ 11
Equity securities and Investments measured at NAV	784	191
	893	202
Less change in net unrealized gains and losses on investments classified as other-than-trading in Net assets without donor restrictions	(3)	(5)
Less change in net unrealized gains and losses on investments in Net assets with donor restrictions	(15)	(7)
Change in net unrealized gains and losses on investments	875	190
Investment income	489	581
Investment income, net	\$ 1,364	\$ 771

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

4. FAIR VALUE MEASUREMENTS

Sutter accounts for certain assets at fair value. A fair value hierarchy for valuation inputs has been established to prioritize the valuation inputs into three levels based on the extent to which inputs used in measuring fair value are observable in the market. Each fair value measurement is reported in one of the three levels, which is determined by the lowest level input that is significant to the fair value measurement in its entirety. These levels are:

Level 1: Quoted prices are available in active markets for identical assets as of the measurement date.

Level 2: Pricing inputs are based on quoted prices for similar instruments in active markets, quoted prices for identical or similar instruments in markets that are not active, and model-based valuation techniques for which all significant assumptions are observable in the market or can be corroborated by observable market data for substantially the full term of the assets.

Level 3: Pricing inputs are generally unobservable for the assets and include situations where there is little, if any, market activity for the investment. The inputs into the determination of fair value require management's judgment or estimation of assumptions that market participants would use in pricing the assets. The fair values are therefore determined using factors that involve judgment and interpretations, including, but not limited to, private and public comparables, third-party appraisals, discounted cash flow models, fund manager estimates and net asset valuations provided by the underlying private investment companies and/or their administrators. Sutter held no Level 3 financial instruments as of December 31, 2025 and 2024.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

4. FAIR VALUE MEASUREMENTS (continued)

The fair value of Sutter's assets measured on a recurring basis consists of the following:

	December 31, 2025		
	Quoted Prices in Active Markets for Identical Instruments (Level 1)	Significant Other Observable Inputs (Level 2)	Total
Liquid investments			
Cash equivalents	\$ 125	\$ —	\$ 125
Equity securities			
U.S. equity	1,656	—	1,656
Foreign equity	824	—	824
Fixed income securities			
U.S. government	372	—	372
U.S. government agencies	—	6	6
U.S. state and local government	—	22	22
U.S. federal agency mortgage-backed	—	762	762
Foreign government	—	274	274
U.S. corporate	470	828	1,298
Foreign corporate	7	143	150
	\$ 3,454	\$ 2,035	\$ 5,489
Investments measured at net asset value			6,864
			<u>\$ 12,353</u>

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

4. FAIR VALUE MEASUREMENTS (continued)

	December 31, 2024		
	Quoted Prices in Active Markets for Identical Instruments (Level 1)	Significant Other Observable Inputs (Level 2)	Total
Liquid investments			
Cash equivalents	\$ 721	\$ —	\$ 721
Equity securities			
U.S. equity	1,417	—	1,417
Foreign equity	566	—	566
Fixed income securities			
U.S. government	454	—	454
U.S. government agencies	—	31	31
U.S. state and local government	—	28	28
U.S. federal agency mortgage-backed	—	634	634
Foreign government	—	181	181
U.S. corporate	17	1,395	1,412
Foreign corporate	6	164	170
	\$ 3,181	\$ 2,433	\$ 5,614
Investments measured at net asset value			4,838
			<u>\$ 10,452</u>

As of December 31, 2025 and 2024, the Level 2 instruments listed in the fair value hierarchy tables above use the following valuation techniques and inputs:

U.S. government agencies securities: The fair value of investments in U.S. government agencies securities classified as Level 2 is primarily determined using consensus pricing methods of observable market-based data. Significant observable inputs include quotes, spreads, and data points for yield curves.

U.S. state and local government securities: The fair value of U.S. state and local government securities classified as Level 2 is determined using a market approach. The inputs include yield benchmark curves, prepayment speeds, and observable market data, such as institutional bids, dealer quotes, and two-sided markets.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

4. FAIR VALUE MEASUREMENTS (continued)

U.S. federal agency mortgage-backed securities: The fair value of U.S. federal agency mortgage-backed securities classified as Level 2 is primarily determined using matrices. These matrices utilize observable market data of bonds with similar features, prepayment speeds, credit ratings, and discounted cash flows. Additionally, observed market movements, tranche cash flows, and benchmark yields are incorporated in the pricing models.

Foreign government and corporate securities: The fair value of investments in foreign government and corporate securities classified as Level 2 is primarily determined using consensus pricing methods of observable market-based data. Significant observable inputs include quotes, bid and ask yields, and issue-specific factors.

U.S. corporate securities: The fair value of investments in U.S. corporate securities classified as Level 2 is primarily determined using techniques that are consistent with the market approach. Significant observable inputs include reported trades, dealer quotes, security-specific characteristics, and multiple sources of spread data points in developing yield curves.

Investments measured at net asset value: Certain investments that are measured using the NAV per share (or its equivalent) practical expedient have not been classified in the fair value hierarchy. The NAV amounts presented in this table are intended to permit reconciliation of the fair value hierarchy to the amounts presented in the Consolidated Balance Sheets.

Certain of the investments are reported using a calculated NAV per share (or its equivalent). These investments are not expected to be sold at amounts that are significantly different from NAV.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

4. FAIR VALUE MEASUREMENTS (continued)

The following tables and explanations identify attributes relating to the nature and risk of such investments:

	December 31, 2025			
	Fair Value	Unfunded Commitments	Redemption Frequency	Redemption Notice Period
Commingled funds – U.S. equity securities	\$ 854	\$ –	Daily	1 day
Commingled funds – foreign equity securities	1,159	–	Semi-monthly, Monthly	5–30 days
Commodity-linked funds	362	–	Daily	None
Hedge funds	3,055	–	Various	10–120 days, None
Private equity funds	889	583	Semi-annually, None	65 days, None
Private equity real estate funds	545	431	Quarterly, None	90 days, None
	<u>\$ 6,864</u>	<u>\$ 1,014</u>		

	December 31, 2024			
	Fair Value	Unfunded Commitments	Redemption Frequency	Redemption Notice Period
Commingled funds – U.S. equity securities	\$ 128	\$ –	Daily	1 day
Commingled funds – foreign equity securities	742	–	Semi-monthly, Monthly	5–30 days
Commodity-linked funds	384	–	Daily	None, 1 day
Hedge funds	2,345	–	Various	10–120 days, None
Private equity funds	705	499	None	None
Private equity real estate funds	534	458	None, Quarterly	None, 90 days
	<u>\$ 4,838</u>	<u>\$ 957</u>		

Commingled funds – U.S. and foreign equity securities: This class includes investments in commingled funds that invest primarily in U.S. or foreign equity securities and attempt to match the returns of specific equity indices. As of December 31, 2025, approximately 42% of this class is redeemable daily with a notice period of 1 day, and approximately 15% of

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

4. FAIR VALUE MEASUREMENTS (continued)

this class is redeemable semi-monthly with a notice period of 5 days. The remaining 43% of this class is redeemable monthly with a notice period of 5 to 30 days.

Commodity-linked funds: This class includes commodity-linked funds that pursue long-only fully collateralized commodity futures strategies to provide diversification and inflation protection. As of December 31, 2025, these funds are redeemable daily with no notice period.

Hedge funds: This class includes investments in hedge funds that expand the universe of potential investment approaches available by employing a variety of strategies and techniques within and across various asset classes. The primary objective for these funds is to balance returns, while limiting volatility by allocating capital to external portfolio managers selected for expertise in one or more investment strategies that may include, but are not limited to, equity long/short, event driven, relative value, and directional. The following summarizes the redemption criteria for the hedge fund portfolio as of December 31, 2025:

% of Hedge Funds	Redemption Criteria	Notice Period
50%	Redeemable monthly	10–120 days
8%	Redeemable quarterly	45–120 days
1%	Redeemable within one year, with monthly gates from 25% to 100%	120 days
5%	Redeemable within one year, with quarterly gates from 25% to 100%	45–120 days
1%	Redeemable within two years, with quarterly gates from 25% to 100%	75 days
9%	Limited to a 12.5% or 25% gate, redeemable quarterly	60–120 days
2%	Limited to a 12.5% gate, redeemable semi-annually	60 days
3%	Redeemable annually	120 days
1%	Six-month lock-up expiring by May 2026	120 days
3%	Twenty-one-month lock-up expiring by March 2026	120 days
10%	Two-year rolling lock-up expiring by December 2027 and quarterly gate limited to 10% annually	60–120 days
1%	Two-year lock-up expiring by December 2027	30–120 days
1%	Three-year rolling lock-up expiring by January 2026	75 days
4%	Redeemable over five years after a rolling annual election with quarterly payments or limited to a 5% quarterly gate	120 days
1%	No redemption allowed until fund dissolves typically from 5 to 8 years	None

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

4. FAIR VALUE MEASUREMENTS (continued)

Private equity funds: This class includes domestic and foreign private equity funds that specialize in providing capital to a variety of investment groups including, but not limited to venture capital, leveraged buyout, mezzanine debt, distressed debt, and other strategies, which may include land, water processing, and alternative energy. As of December 31, 2025, 1% of this class is redeemable semi-annually after a two-year lock-up expiring in May 2027, with a notice period of at least 65 days. There is no provision for redemptions during the life of these funds for the remaining 99%. These funds are typically held from 8 to 17 years.

Private equity real estate funds: This class includes domestic and foreign investments in real estate that are held in limited partnership funds, joint ventures, and other investments comprised of retail, office, industrial, and multi-family properties. As of December 31, 2025, 2% of this class is redeemable quarterly, with a notice period of at least 90 days. There is no provision for redemptions during the life of these funds for the remaining 98%. These funds are typically held from 8 to 11 years.

5. PROPERTY PLANT AND EQUIPMENT

Property, plant and equipment consist of the following:

	December 31,	
	2025	2024
Land improvements	\$ 218	\$ 218
Leasehold improvements	1,174	822
Buildings and improvements	10,840	10,143
Equipment	4,805	4,473
	17,037	15,656
Less amortization and accumulated depreciation	(9,772)	(9,241)
	7,265	6,415
Land	766	622
Construction-in-progress	773	522
	<u>\$ 8,804</u>	<u>\$ 7,559</u>

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

6. OTHER ASSETS

Other current assets consist of the following:

	December 31,	
	2025	2024
Hospital fee program receivable	\$ 438	\$ 396
Prepaid expenses	241	223
Inventories	157	147
Other	638	404
	<u>\$ 1,474</u>	<u>\$ 1,170</u>

Other non-current assets consist of the following:

	December 31,	
	2025	2024
Right-of-use assets for operating leases	\$ 802	\$ 770
Hospital fee program receivable	—	445
Goodwill, net	173	153
Trust receivable	98	84
Investments in joint ventures	191	106
Prepaid postretirement benefits	83	24
Other	268	235
	<u>\$ 1,615</u>	<u>\$ 1,817</u>

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

7. LONG-TERM OBLIGATIONS

Long-term obligations consist of the following:

	December 31,	
	2025	2024
Tax-exempt revenue bonds under the Sutter Health Master Indenture of Trust, fixed interest at 4.0% to 5.0%, through 2048 (includes unamortized premiums of \$37 and \$62 and debt issuance costs of (\$2) and (\$3) at December 31, 2025 and 2024, respectively)	\$ 1,504	\$ 1,560
Taxable bonds under the Sutter Health Master Indenture of Trust, fixed interest at 2.29% to 5.55%, through 2053 (includes unamortized discount of (\$1) and (\$1) and debt issuance costs of (\$16) and (\$12) at December 31, 2025 and 2024, respectively)	4,410	3,414
Various collateralized and unsecured obligations	272	26
Obligations under finance leases	211	205
	6,397	5,205
Less current portion	(43)	(347)
	\$ 6,354	\$ 4,858

The central financing vehicle for Sutter is the Obligated Group. Those entities that comprise the Obligated Group are each nonprofit public benefit corporations organized and existing under the laws of the State of California. Only the Obligated Group members are subject to the covenants under the Master Trust Indenture, dated as of October 1, 2020. Each Obligated Group member has granted to the Master Trustee a security interest in its gross receivables to secure the timely payment and performance of its covenants under the Master Trust Indenture.

In May 2025, \$1,300 of taxable bonds Series 2025A were issued by Sutter Health. The proceeds are to be used for general corporate purposes.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

7. LONG-TERM OBLIGATIONS (continued)

In 2025, Sutter entered into a taxable commercial paper program. Under the program, Sutter is registered to issue up to \$200. At December 31, 2025, \$200 of commercial paper with a discount rate of 3.87% to 4.02% was outstanding with maturities generally less than nine months and is included in Short-term borrowings on the Consolidated Balance Sheets. The discount on issuance is recognized as interest expense over the term of the note.

At December 31, 2025 Sutter Health maintained a syndicated line of credit with banks aggregating to \$700. As of December 31, 2025, \$253 is outstanding on this credit facility.

Aggregate principal payments of long-term obligations, excluding finance leases, various collateralized and unsecured obligations, net unamortized premiums and discounts, and issuance costs as of December 31, 2025, are as follows:

2026	\$	34
2027		15
2028		357
2029		22
2030		723
Thereafter		4,745
	\$	<u>5,896</u>

Sutter paid interest of \$210 and \$200 for the years ended December 31, 2025 and 2024, respectively.

8. LEASES

Sutter leases property and equipment under operating and finance leases. The related assets and obligations are recorded at the present value of lease payments over the term of the agreements. Many of Sutter's leases include rental escalation clauses, renewal options, and/or termination options that are factored into the determination of lease payments. Variable lease payments are non-lease services related to the lease, are excluded from the Right-of-use assets and lease liabilities, and are recognized in the period in which the obligation of those payments is incurred. Generally, Sutter does not include renewal options in the lease terms for calculating the lease liability, as Sutter maintains operational flexibility and is not reasonably certain the renewal options will be

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

8. LEASES (continued)

exercised. Most of Sutter's leases do not provide a readily determinable implicit rate in the contract; therefore, the incremental borrowing rate is estimated to discount the lease payments based on information available at lease commencement.

Sutter elected the package of practical expedients permitted under the transition guidance which, among other things, allowed the historical lease classification not to be reassessed. Sutter made an accounting policy election not to apply the recognition requirements of the guidance to short-term leases with a term of 12 months or less for equipment and vehicle classes of assets. Sutter also made an accounting policy election not to separate non-lease components from lease components for all classes of assets. Sutter did not elect the hindsight practical expedient, which permits entities to use hindsight in determining the lease term and assessing impairment.

Lease-related assets and liabilities are recorded on the Consolidated Balance Sheets as follows:

		December 31,	
Classification		2025	2024
Right-of-use Assets			
Operating	Other non-current assets	\$ 802	\$ 770
Finance	Property, plant and equipment, net	167	164
		<u>\$ 969</u>	<u>\$ 934</u>
Current Liabilities			
Operating	Other accrued expenses	\$ 120	\$ 103
Finance	Current portion of long-term obligations	5	8
Non-current Liabilities			
Operating	Other	820	776
Finance	Long-term obligations, less current portion	206	197
		<u>\$ 1,151</u>	<u>\$ 1,084</u>

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

8. LEASES (continued)

	December 31,	
	2025	2024
Weighted-average operating leases remaining lease term	17.9 years	18.4 years
Weighted-average finance leases remaining lease term	22.6 years	23.9 years
Weighted-average operating lease discount rate	4.7%	4.6%
Weighted-average finance lease discount rate	4.6%	4.6%

The components of lease costs are as follows:

	Year ended December 31,	
	2025	2024
Operating lease cost	\$ 139	\$ 125
Variable lease cost	\$ 39	\$ 35
Finance lease cost:		
Amortization of leased assets	\$ 15	\$ 16
Interest on lease liabilities	\$ 10	\$ 9

Supplemental cash flow information related to leases is as follows:

	Year ended December 31,	
	2025	2024
Cash paid for amounts included in the measurement of lease liabilities:		
Operating cash outflows for operating leases	\$ 157	\$ 129
Operating cash outflows for finance leases	\$ 10	\$ 9
Financing cash outflows for finance leases	\$ 11	\$ 12
Right-of-use assets obtained in exchange for lease obligations:		
Operating leases	\$ 105	\$ 217
Finance leases	\$ 3	\$ —

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

8. LEASES (continued)

Future lease payments as of December 31, 2025 for operating and finance leases are as follows:

	Operating Leases	Finance Leases
2026	\$ 158	\$ 14
2027	150	14
2028	135	15
2029	118	15
2030	86	16
Thereafter	834	278
	1,481	352
Less imputed interest	(541)	(141)
	<u>\$ 940</u>	<u>\$ 211</u>

9. NET ASSETS AND CONTRIBUTIONS

Sutter Health and its not-for-profit affiliates receive donations from generous individuals and organizations that support certain programs and services. Donations included in Net assets with donor restrictions were maintained for the following purposes:

	December 31, 2025	2024
Subject to expenditure for specified purpose:		
Capital projects and medical equipment	\$ 29	\$ 50
Research and education	67	67
Operations and/or capital projects	374	314
	470	431
Subject to passage of time	74	76
Subject to donor restrictions in conjunction with Sutter's spending policy:		
Investment in perpetuity – endowment	160	153
	<u>\$ 704</u>	<u>\$ 660</u>

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

9. NET ASSETS AND CONTRIBUTIONS (continued)

From time to time, Sutter Health's or its not-for-profit affiliate's board of directors will designate certain unrestricted funds to be used in the future for specific projects. Board-designated funds included in Net assets without donor restrictions were maintained for the following purposes:

	December 31,	
	2025	2024
Capital projects and medical equipment	\$ 153	\$ 136
Research and education	29	32
Operations and/or capital projects	219	186
	<u>\$ 401</u>	<u>\$ 354</u>

Sutter Health and its not-for-profit affiliates report individuals' and organizations' unconditional promises to give cash or other assets at fair value at the date Sutter Health or its not-for-profit affiliate receives the promises. Sutter Health and its not-for-profit affiliates report conditional promises to give and conditional indications of intentions to give at fair value when the conditions are met. Therefore, Sutter Health and its not-for-profit affiliates do not recognize any revenue or receivable at the time a conditional promise or indication of intent is received. Sutter Health and its not-for-profit affiliates' conditional promises or indications of intent received were \$339 and \$319 for the years ended December 31, 2025 and 2024, respectively. These gifts will support clinical programs and technology.

Endowments: Sutter Health and its not-for-profit affiliates follow the Uniform Prudent Management of Institutional Funds Act (UPMIFA). UPMIFA eliminates the concept of "historic dollar value" and allows an institution to spend or accumulate as the board determines is prudent for the uses, benefits, purposes, and duration of the endowment fund, unless the gift instrument states a particular spending rate or formula.

In accordance with UPMIFA, Sutter Health and its not-for-profit affiliates consider the following factors when appropriating or accumulating an endowment fund: (i) general economic conditions, (ii) effects of inflation and deflation, (iii) the purposes of the institution and the endowment fund, (iv) expected total return from income and appreciation of investments, (v) Sutter's other resources, (vi) the duration and preservation of the endowment fund, and (vii) Sutter's investment policies.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

9. NET ASSETS AND CONTRIBUTIONS (continued)

If the fair market value of assets associated with individual endowment funds falls below the corpus, Sutter Health or its not-for-profit affiliate's management assesses facts and circumstances to determine whether to suspend appropriation activities until the corpus has recovered or to continue to withdraw funds in compliance with UPMIFA in order to fund critical initiatives. There were no deficiencies of this nature reported in Net assets with donor restrictions as a result of unfavorable investment market fluctuations as of December 31, 2025 and 2024.

Following UPMIFA, Sutter Health and its not-for-profit affiliates' investment and spending policies for endowment assets attempt to provide a predictable stream of funding to programs supported by its endowment, while seeking to maintain the purchasing power of the endowment assets. Under these policies, endowment assets are invested in a manner that is intended to produce results that exceed the price and yield results, while assuming a moderate level of investment risk.

To satisfy its long-term rate-of-return objectives, Sutter relies on a balanced investment strategy in which investment returns are achieved through both capital appreciation (realized and unrealized) and current yield (interest and dividends). Sutter targets a diversified asset allocation that places a greater emphasis on equity-based investments to achieve its long-term return objectives within prudent risk constraints.

As of December 31, 2025 and 2024, the endowment net assets for board-designated funds totaled \$181 and \$157, respectively, and are classified as without donor restrictions. The endowment net assets for donor-restricted endowment funds totaled \$261 and \$232 as of those same dates and are classified as with donor restrictions.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

9. NET ASSETS AND CONTRIBUTIONS (continued)

The changes in endowment net assets are as follows:

	Net Assets without Donor Restrictions	Net Assets with Donor Restrictions	Total
Balance at December 31, 2023	\$ 145	\$ 205	\$ 350
Investment return, net	15	20	35
Contributions	—	14	14
Appropriation of endowment assets for expenditure	(3)	(3)	(6)
Other	—	(4)	(4)
Balance at December 31, 2024	\$ 157	\$ 232	\$ 389
Investment return, net	29	30	59
Contributions	—	4	4
Appropriation of endowment assets for expenditure	(5)	(4)	(9)
Other	—	(1)	(1)
Balance at December 31, 2025	\$ 181	\$ 261	\$ 442

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

10. REVENUES

Sutter records revenue in four financial statement categories: Patient service revenues, Premium revenues, Contributions, and Other. Performance obligations are identified based on the nature of the services provided.

Sutter elected the practical expedient and does not adjust the promised amount of consideration for the effects of a significant financing component, due to Sutter's expectation that the period between the time the service is provided and the receipt of payment aligns with customary billing and collection practices in the normal course of business. However, Sutter does, in certain instances, enter into payment agreements that allow payments in excess of one year. For those cases, the financing component is not deemed to be significant to the contract. Additionally, Sutter elected to apply the optional exemption, because all of its performance obligations relate to contracts with a duration of less than one year. Therefore, Sutter is not required to disclose the aggregate amount of the transaction price allocated to performance obligations that are unsatisfied or partially satisfied at the end of the reporting period.

Patient service revenues: Sutter's Patient service revenues are reported at the amount that reflects the consideration to which Sutter expects to be paid for providing patient care. These amounts are due from patients and third-party payers, including health insurers and government programs. Patients who meet Sutter's criteria for charity care are provided care without charge or at amounts less than established rates. Such amounts determined to qualify as charity care are not reported as revenue. Generally, Sutter bills patients and third-party payers after services are performed.

Patient service revenues are recognized as performance obligations are satisfied. Inpatient services are performance obligations satisfied over time, and revenue is recognized based on actual charges incurred in relation to total expected or actual charges. Unsatisfied or partially unsatisfied performance obligations relate to inpatient acute care services at the end of the reporting period. The performance obligations for these contracts are generally completed when the patients are discharged, which generally occurs within days or weeks of the end of the reporting period. Outpatient services are performance obligations satisfied at a point in time, and revenue is recognized when goods or services are provided and Sutter does not believe it is required to provide additional goods or services.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

10. REVENUES (continued)

Sutter uses a portfolio approach to account for categories of patient contracts as a collective group, rather than recognizing revenue on an individual contract basis. The portfolios consist of major payer classes for inpatient and outpatient revenue. Based on historical collection trends, Sutter believes that revenue recognized by utilizing the portfolio approach approximates the revenue that would have been recognized if an individual contract approach had been used.

The health care industry is subject to voluminous and complex laws and regulations of federal, state and local governments. Compliance with such laws and regulations can be subject to future government review and interpretation, as well as regulatory actions unknown or unasserted at this time. These laws and regulations include, but are not necessarily limited to, matters such as licensure, accreditation, government health care program participation requirements, reimbursement laws and regulations, anti-kickback and anti-referral laws and false claims prohibitions and, in the case of tax-exempt affiliates, the requirements of tax exemption. Sutter Health operates an Ethics and Compliance Program, which reviews compliance with government health care program requirements and investigates allegations of non-compliance received from internal and external sources. From time to time, findings may result in repayment of monies previously received from government, other third-party payers and/or disclosure of such overpayments including, but not limited to, disclosure to the Centers for Medicare & Medicaid Services (CMS) and its contracted agents, or the Office of Inspector General, Department of Health and Human Services. As a result, there is at least a reasonable possibility that the recorded estimates may change by a material amount in the near term.

The majority of Sutter's services are provided to patients with third-party coverage, and Sutter has agreements with third-party payers that provide for payments to Sutter at contractually adjusted amounts. Patient service revenues are estimated based on the terms of the contractual agreement with the payer, Sutter's historical settlement activity, and other information. Settlements with third-party payers for retroactive adjustments due to audits, reviews, or investigations are considered variable consideration and are included in the determination of Patient service revenues when information becomes available.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

10. REVENUES (continued)

Payment arrangements are as follows:

Medicare: Inpatient acute care services and outpatient services provided to Medicare program beneficiaries are paid at prospectively determined rates per diagnosis. Sutter is paid for cost-reimbursable items at a tentative rate. Physician services are paid based upon established fee schedules. Outpatient services are paid using prospectively determined rates. Amounts received from the Medicare programs are subject to audit and final settlement by a Medicare Administrative Contractor after submission of annual cost reports. Sutter's Medicare cost reports have been audited generally through December 31, 2019. The estimated net settlement balances and adjustments from the finalization of prior-year cost reports were immaterial in 2025 and 2024.

Medi-Cal: Inpatient and outpatient services provided to Medi-Cal program beneficiaries are paid either under contracted rates or cost-reimbursable items at a tentative rate. Services are generally paid at prospectively determined rates per discharge, per occasion of service, or per covered member. Amounts received from Medi-Cal programs are subject to audit and final settlement by the California Department of Health Care Services after submission of annual cost reports. Sutter's Medi-Cal cost reports have been audited generally through December 31, 2020. The estimated net settlement balances and adjustments from the finalization of prior-year cost reports were immaterial in 2025 and 2024.

Commercial: Inpatient and outpatient services provided to patients covered under commercial insurance policies are paid using a variety of payment methodologies based on contractual agreements. The transaction price for commercial payers is reduced by explicit contractual adjustments and implicit price concessions based on collection history with this portfolio of patients.

Other: Inpatient and outpatient services provided to patients, not covered by third-party payers, are paid based on Sutter's policies and the patient's ability to pay. Sutter reduces the transaction price by implicit price concessions to uninsured patients and patients with uninsured balances, such as copays and deductibles. The implicit price concessions included in estimating the transaction price represent the difference between amounts billed to patients and the amounts Sutter expects to collect based on its collection history with this portfolio of patients. Subsequent

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

10. REVENUES (continued)

changes to the estimates are considered variable consideration and are included in Patient service revenues when information becomes available.

As part of its Patient service revenues analysis, Sutter examines the fluctuations in payer, geographical area and lines of business, as each factor represents a varying degree of uncertainty regarding the nature, timing, and extent of payments.

The composition of Patient service revenues by payer is as follows:

	Year ended December 31,	
	2025	2024
Medicare	\$ 4,592	\$ 4,209
Medi-Cal	1,652	2,116
Commercial	10,462	9,280
Other	372	207
	<u>\$ 17,078</u>	<u>\$ 15,812</u>

The composition of Patient service revenues, based on Sutter's lines of business, is as follows:

	Year ended December 31,	
	2025	2024
Acute Care	\$ 10,348	\$ 9,797
Medical Foundation	6,236	5,449
Other	1,098	1,015
Eliminations	(604)	(449)
	<u>\$ 17,078</u>	<u>\$ 15,812</u>

The State of California enacted legislation for a hospital fee program to fund certain Medi-Cal coverage expansions. The program charges certain hospitals a quality assurance fee that is used to obtain federal matching funds for Medi-Cal, with the proceeds redistributed as supplemental payments to California hospitals that treat Medi-Cal

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

10. REVENUES (continued)

patients. There are four approved hospital fee programs that had activity in 2025 and/or 2024: a 30-month hospital fee program covering the period from January 1, 2017 through June 30, 2019, a 30-month hospital fee program covering the period from July 1, 2019 through December 31, 2021, a 12-month hospital fee program covering the period from January 1, 2022 through December 31, 2022, and a 24-month hospital fee program covering the period from January 1, 2023 through December 31, 2024. Supplemental payments met all criteria related to revenue recognition, and the quality assurance fees are both probable and estimable. Accordingly, all related supplemental payments have been recognized as revenue and related quality assurance fees recognized as expense in 2025 and 2024.

In 2024, a 12-month hospital fee program was established covering the period January 1, 2025 through December 31, 2025. In 2025 the application was submitted to CMS for approval. In January 2026, CMS granted the Department of Health Care Services (DHCS) a 60-day extension to resubmit the application. The program's revenue is fully constrained, and the revenue and expense are not recognized in these financial statements as of December 31, 2025 and the period then ended.

Patient service revenues and Other expenses include amounts for the hospital fee program as follows:

	Year ended December 31,	
	2025	2024
Hospital fee program revenue	\$ 23	\$ 664
Hospital fee program expense	—	(403)
Income from operations from hospital fee program	<u>\$ 23</u>	<u>\$ 261</u>

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

10. REVENUES (continued)

Hospital fee program assets and liabilities are recorded on the Consolidated Balance Sheets as follows:

	December 31,	
	2025	2024
Other current assets	\$ 438	\$ 396
Other non-current assets	–	445
	<u>\$ 438</u>	<u>\$ 841</u>
Accounts payable	\$ 199	\$ 203
Other non-current liabilities	–	204
	<u>\$ 199</u>	<u>\$ 407</u>

Premium revenues: Sutter has entered into payment agreements with certain commercial insurance carriers, health maintenance organizations, preferred provider organizations, and members of individual and family plans and subscribing employers for small and large cap coverage. The basis for payment to Sutter, under these agreements, includes capitated arrangements, prospectively determined rates per diagnosis, prospectively determined daily rates, rates by demographics, and rates by a number of factors, including experience. The transaction price may be reduced by discounts, reinsurance premiums, and implicit price concessions based on collection history. Other adjustments may include prior year settlements, stop-loss recoveries, ceded premiums and risk adjustment factors. Performance obligations are satisfied over the passage of time by standing ready to provide services.

Settlements with third-party payers for retroactive adjustments are considered variable consideration and are included in the determination of Premium revenues when information becomes available. Adjustments arising from a change in the transaction price and from the finalization of prior-year settlements were immaterial for the years ended December 31, 2025 and 2024.

As part of its Premium revenues analysis, Sutter examines the fluctuations in geographical area and lines of business, as each factor represents a varying degree of uncertainty regarding the nature, timing and extent of payments. Sutter's premium revenue is reported at an amount that reflects the consideration to which Sutter expects to be paid.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

10. REVENUES (continued)

The composition of Premium revenues, based on Sutter's lines of business, is as follows:

	Year ended December 31,	
	2025	2024
Acute Care	\$ 908	\$ 793
Medical Foundation	897	853
Insurance	966	826
Other	216	182
Eliminations	(779)	(645)
	<u>\$ 2,208</u>	<u>\$ 2,009</u>

Contributions: Sutter receives contributions and grants from donors and government agencies. Sutter reported \$118 and \$4 as contributions from the Federal Emergency Management Agency for the years ended December 31, 2025 and 2024, respectively.

Other revenues: Sutter has additional revenue streams from health professionals, rental properties and parking. Revenue is recognized when obligations under the terms of the contract are satisfied. Revenues from these services are measured as the amount of consideration Sutter expects to receive for those services.

11. COMMUNITY BENEFIT EXPENSE

In 2025, Sutter invested \$1,191 in community benefit programs that help improve the overall health and wellbeing of the regions Sutter serves.

Services for the poor and underserved include traditional charity care, unpaid costs of public programs treating Medi-Cal, county, and indigent beneficiaries, other services for the poor and underserved, and cash donations towards programs and services for the underserved. Charity care covers health care services provided to persons who meet certain criteria and cannot afford to pay. Sutter provided charity care services to patients at an estimated cost of \$101 and \$96 for 2025 and 2024, respectively.

Benefits for the broader community include costs of providing the following services: health screenings and other health-related services, training health professionals,

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

11. COMMUNITY BENEFIT EXPENSE (continued)

educating the community with various seminars and classes, the cost of performing medical research, and the costs associated with providing free clinics and community services. Benefits for the broader community also include contributions Sutter makes to community agencies to fund charitable activities.

The following is a summary of Sutter's estimated costs of providing services to the poor and broader community for the year ended December 31, 2025 (unaudited):

Services for the poor and underserved

Traditional charity care	\$	101
Unpaid costs of public programs:		
Medi-Cal		830
Other public programs		53
Other benefits for the poor and underserved		67
		1,051

Benefits for the broader community

Nonbilled services		35
Education and research		101
Cash and in-kind donations		2
Other community benefits		2
		140
	\$	1,191

12. POSTRETIREMENT BENEFITS

Sutter sponsors and participates in various employee benefit plans, including a noncontributory defined benefit plan (the "Retirement Plan"), a noncontributory defined contribution plan, and several contributory defined contribution plans. In addition, certain affiliates participate in multiemployer defined benefit retirement plans. Sutter's total net postretirement benefits were \$338 and \$245 in 2025 and 2024, respectively.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

Sutter's measurement date for plan assets, pension obligations and net periodic pension cost associated with the Retirement Plan is December 31. The changes in benefit obligations and plan assets for the Retirement Plan are as follows:

	Year ended December 31,	
	2025	2024
Projected benefit obligation at beginning of year	\$ 5,593	\$ 5,600
Service cost	235	236
Interest cost	329	302
Actuarial loss (gain)	126	(269)
Benefits paid	(323)	(276)
Projected benefit obligation at measurement date	<u>\$ 5,960</u>	<u>\$ 5,593</u>
Fair value of plan assets at beginning of year	\$ 5,595	\$ 5,474
Actual gain on plan assets	733	397
Benefits paid	(323)	(276)
Fair value of plan assets at measurement date	<u>\$ 6,005</u>	<u>\$ 5,595</u>
Funded status at end of year	<u>\$ 45</u>	<u>\$ 2</u>

The accumulated benefit obligation for the Retirement Plan was \$5,357 and \$5,021 as of December 31, 2025 and 2024, respectively.

The actuarial loss of \$126 for the year ended December 31, 2025, was primarily due to the decrease in discount rate of 5.7% in 2025 from 5.8% in 2024, updated cash balance interest crediting rates and census data that differed from expectations. This was offset by changes in year-end census data adjustments and an updated annuity conversion interest rate. The actuarial gain of \$269 for the year ended December 31, 2024, was primarily due to the increase in discount rate of 5.8% in 2024 from 5.3% in 2023 and updated cash balance interest crediting rates. This was offset by changes in census data that differed from expectations and an updated annuity conversion interest rate.

Included in Controlling interests net assets without donor restrictions as of December 31, 2025 and 2024, are the following amounts that have not yet been

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

recognized in net periodic benefit cost: unrecognized prior service credits of \$161 and \$185, respectively, and unrecognized actuarial losses of \$82 and \$330, respectively.

The benefits expected to be paid from the Retirement Plan as of December 31, 2025 in each of the next five years, and in the aggregate for the next five years, are as follows:

2026	\$	329
2027		355
2028		374
2029		394
2030		409
2031–2035		2,299
	\$	<u>4,160</u>

The actuarial assumptions used by the Retirement Plan are as follows:

	December 31,	
	2025	2024
Weighted-average discount rates for calculating pension expense	5.8%	5.3%
Weighted-average discount rates for calculating projected benefit obligation	5.7%	5.8%
Weighted-average rates of compensation increase for calculating pension expense	4.5%–6.0%	4.5%–6.5%
Weighted-average rates of compensation increase for calculating projected benefit obligation	4.5%–5.0%	4.5%–6.0%
Weighted-average interest crediting rates for calculating projected benefit expense	4.5%–4.7%	4.0%–4.8%
Weighted-average interest crediting rates for calculating pension obligation	4.5%–4.8%	4.5%–4.7%
Expected long-term rates of return on plan assets for calculating pension expense	6.6%	7.4%

As of December 31, 2025 and 2024, the healthy mortality assumption reflected the Pri-2012 table, and the mortality projection scale reflected the MP-2021 without adjustment.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

The components of the Retirement Plan's net periodic benefit cost are as follows:

	Year ended December 31,	
	2025	2024
Service cost	\$ 235	\$ 236
Interest cost	329	302
Expected return on plan assets	(359)	(393)
Amortization of actuarial loss	–	4
Amortization of prior service credit	(23)	(23)
	<u>\$ 182</u>	<u>\$ 126</u>

In addition to the Retirement Plan, Sutter also has noncontributory postretirement health benefit plans (the "Health Plans"). Sutter's measurement date for plan assets, retiree medical obligations and net periodic retiree medical cost associated with the Health Plans is December 31. The changes in benefit obligations for the Health Plans are as follows:

	Year ended December 31,	
	2025	2024
Projected benefit obligation at beginning of year	\$ 254	\$ 281
Service cost	10	11
Interest cost	15	14
Actuarial loss (gain)	1	(36)
Benefits paid	(17)	(16)
Projected benefit obligation at measurement date	<u>\$ 263</u>	<u>\$ 254</u>
Fair value of plan assets at beginning of year	\$ 276	\$ 268
Actual gain on plan assets	37	20
Employer contributions	5	4
Benefits paid	(17)	(16)
Fair value of plan assets at measurement date	<u>\$ 301</u>	<u>\$ 276</u>
Funded status at end of year	<u>\$ 38</u>	<u>\$ 22</u>

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

The actuarial loss of \$1 for the year ended December 31, 2025, was primarily due to discount rate decreases to 5.5% in 2025 from ranges of 5.7% to 5.8% in 2024 and updated trend. This was offset by changes in data that differed from expectations and update to claims. The actuarial gain of \$36 for the year ended December 31, 2024, was primarily due to discount rate increases to ranges of 5.7% to 5.8% in 2024 from 5.2% in 2023, updated drawdown assumption and trend, and changes in census data that differed from expectations.

Included in Controlling interests net assets without donor restrictions as of December 31, 2025 and 2024, are the following amounts that have not yet been recognized in net periodic benefit cost: unrecognized prior service costs of \$6 and \$8, respectively, and unrecognized actuarial gain of \$101 and \$87, respectively.

The benefits expected to be paid from the Health Plans as of December 31, 2025 in each of the next five years, and the next following five years, are as follows:

2026	\$	19
2027		20
2028		21
2029		22
2030		22
2031–2035		120
	\$	<u>224</u>

The actuarial assumptions used by the Health Plans are as follows:

	December 31,	
	2025	2024
Weighted-average discount rates for calculating retiree medical expense	5.7%–5.8%	5.2%
Weighted-average discount rates for calculating projected benefit obligation	5.5%	5.7%–5.8%
Expected long-term rates of return on plan assets for calculating retiree medical expense	6.6%	7.4%

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

The components of the Health Plans' net periodic benefit cost are as follows:

	Year ended December 31,	
	2025	2024
Service cost	\$ 10	\$ 11
Interest cost	15	14
Expected return on plan assets	(18)	(18)
Amortization of prior service cost	1	—
Amortization of actuarial gain	(4)	(2)
	<u>\$ 4</u>	<u>\$ 5</u>

Sutter's projected medical cost trend rate related to the Health Plans for 2025 is 6.8%. The assumed medical cost trend rate is expected to gradually decrease in subsequent years to 4.8% in 2034 and thereafter.

The Sutter Health Retirement Benefits Investment Committee oversees the investments and investment policy of the plans. Management of the assets is governed by the application of modern portfolio theory, resulting in asset class diversification and mean-variance optimization. Sutter's investment strategy is to balance the liquidity needs of the plans with the long-term return goals necessary to satisfy future obligations.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

The target asset allocation seeks to reduce volatility while capturing the equity premium from the capital markets over the long term and maintaining security of principal to meet near-term expenses and obligations. The target asset allocation at December 31, 2025, by major asset category, is as follows:

	Target Allocation
Equity securities	40%
Fixed income securities	18%
Other investments – alternative	32%
Real estate investments	10%
	<u>100%</u>

Equity securities are comprised of U.S. and foreign equity securities, common and collective trusts, and commingled funds. The equity securities' target asset allocation of 40.0% is further comprised of 17.7% domestic large capitalization, 4.2% domestic small capitalization and 18.1% international/global.

The expected long-term rate of return assumptions of 6.6% and 7.4% for 2025 and 2024, respectively, are based on capital market assumptions rate of return on comparative market indices for the major asset classes represented in the portfolio, net of administrative expenses.

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

A fair value hierarchy has been established, with three levels that prioritize the valuation inputs into each level (see Note 4). The fair value and NAV of the Retirement Plan's and the Health Plans' assets measured on a recurring basis consist of the following:

	December 31, 2025			
	Quoted Prices in Active Markets for Identical Instruments (Level 1)	Significant Other Observable Inputs (Level 2)	Net Asset Value (NAV)	Total
Liquid investments:				
Cash equivalents	\$ 4	\$ —	\$ 52	\$ 56
Equity securities:				
U.S. equity	789	—	—	789
Foreign equity	469	—	—	469
Common collective trusts	—	—	295	295
Commingled funds	—	—	947	947
Fixed income securities:				
U.S. government and agencies	197	4	—	201
U.S. federal agency mortgage-backed	—	208	—	208
Foreign government	—	112	—	112
U.S. corporate	173	227	—	400
Foreign corporate	—	54	—	54
Other investments:				
Private equity funds	—	—	720	720
Private equity real estate funds	—	—	697	697
Hedge funds	—	—	1,280	1,280
Receivables for securities sold	501	—	—	501
Payables for securities sold	(434)	—	—	(434)
Accrued income	11	—	—	11
	\$ 1,710	\$ 605	\$ 3,991	\$ 6,306

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

	December 31, 2024			
	Quoted Prices in Active Markets for Identical Instruments (Level 1)	Significant Other Observable Inputs (Level 2)	Net Asset Value (NAV)	Total
Liquid investments:				
Cash equivalents	\$ 12	\$ —	\$ 296	\$ 308
Equity securities:				
U.S. equity	802	—	—	802
Foreign equity	435	—	—	435
Common collective trusts	—	—	265	265
Commingled funds	—	—	673	673
Fixed income securities:				
U.S. government and agencies	165	2	—	167
U.S. federal agency mortgage-backed	—	166	—	166
Foreign government	—	81	—	81
U.S. corporate	—	348	—	348
Foreign corporate	—	42	—	42
Other investments:				
Private equity funds	—	—	690	690
Private equity real estate funds	—	—	726	726
Hedge funds	—	—	1,185	1,185
Receivables for securities sold	262	—	—	262
Payables for securities sold	(289)	—	—	(289)
Accrued income	10	—	—	10
	\$ 1,397	\$ 639	\$ 3,835	\$ 5,871

Securities Lending: Retirement Plan assets may also be loaned to various brokers in exchange for securities as collateral. As of December 31, 2025 and 2024, the fair value of securities on loan is \$99 and \$53, respectively, and the related noncash collateral is \$104 and \$54, respectively. The majority of the securities on loan are U.S. equity securities and U.S. government securities and are collateralized by U.S. government securities.

Certain affiliates participate in multiemployer defined benefit retirement plans. The risks of participating in multiemployer plans are different from single-employer plans in the following aspects: (i) assets contributed to the multiemployer plan by one employer may

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

12. POSTRETIREMENT BENEFITS (continued)

be used to provide benefits to employees of other participating employers, (ii) if a participating employer stops contributing to the plan, the unfunded obligations of the plan may be borne by the remaining participating employers, and (iii) if the affiliates choose to stop participating in the multiemployer plan, the affiliates may be required to pay the plan an amount based on the underfunded status of the plan, referred to as a withdrawal liability. Sutter's contributions to such plans were \$27 and \$17 in 2025 and 2024, respectively.

Sutter also maintains various defined contribution plans for eligible employees. Sutter's contributions to such plans were \$125 and \$97 in 2025 and 2024, respectively.

13. FUNCTIONAL CLASSIFICATION OF EXPENSES

Sutter groups like expenses into financial statement lines and classifies programmatic expenses by business line. Expenses that are attributable to one or more programs or supporting functions are allocated based on expenses, square footage, and other criteria.

The following is a functional classification of Sutter's Total expenses:

	Year ended December 31, 2025						
	Program				General and Administrative	Total	
	Medical						
	Acute Care	Foundation	Other	Eliminations			
Salaries and employee benefits	\$ 5,305	\$ 1,856	\$ 1,111	\$ (403)	\$ 972	\$	8,841
Purchased services	2,997	4,249	864	(2,838)	370		5,642
Supplies	1,758	720	237	(33)	6		2,688
Depreciation and amortization	518	199	91	(63)	10		755
Interest	169	40	4	(85)	85		213
Other	326	248	1,297	(891)	192		1,172
	<u>\$ 11,073</u>	<u>\$ 7,312</u>	<u>\$ 3,604</u>	<u>\$ (4,313)</u>	<u>\$ 1,635</u>	<u>\$</u>	<u>19,311</u>

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

13. FUNCTIONAL CLASSIFICATION OF EXPENSES (continued)

Year ended December 31, 2024							
	Program				General and Administrative		Total
	Acute Care	Medical Foundation	Other	Eliminations			
Salaries and employee benefits	\$ 4,842	\$ 1,692	\$ 958	\$ (369)	\$ 935	\$	8,058
Purchased services	2,741	3,621	804	(2,464)	423		5,125
Supplies	1,512	639	218	(29)	6		2,346
Depreciation and amortization	516	176	93	(92)	9		702
Interest	110	25	2	—	41		178
Other	711	218	1,279	(762)	210		1,656
	<u>\$ 10,432</u>	<u>\$ 6,371</u>	<u>\$ 3,354</u>	<u>\$ (3,716)</u>	<u>\$ 1,624</u>	<u>\$</u>	<u>18,065</u>

14. CONTINGENCIES AND COMMITMENTS

Contingencies: From time to time, Sutter receives and responds to investigations and requests concerning possible violations of reimbursement, false claims, anti-kickback and anti-referral statutes, environmental regulations, and other regulations by health care providers from federal and state regulatory agencies, including, but not limited to, CMS, Health Resources & Services Administration, the U.S. Department of Justice (DOJ), the California Attorney General, and the California Department of Public Health. Sutter is also involved in litigation such as medical professional liability and contractual disputes, as both plaintiff and defendant, and other routine labor matters, proposed class action complaints, tax examinations, security events resulting in potential privacy incidents, internal compliance activities (including those discussed in Note 10), and regulatory investigations and examinations arising in the ordinary course of business. Based on Sutter's assessment of the matters, the uncertainty of litigation, and the preliminary stages of many of the matters, Sutter cannot estimate the reasonable possible loss or range of loss that may result from these matters, except as stated in the consolidated financial statements, including this note. However, there can be no assurance that the resolution of any of these matters will not have a material adverse effect on Sutter's consolidated financial position or results of operations. Following is a discussion of a matter of note.

A plaintiff filed a civil class action lawsuit against Sutter Health and certain affiliates, alleging violations of antitrust law arising out of, among other things, Sutter Health and the affiliates' arrangements with health plans. On March 11, 2022, following a jury trial, the jury returned a unanimous verdict in favor of Sutter Health and the affiliates. Plaintiffs

Sutter Health and Affiliates

Notes to Consolidated Financial Statements (continued)

(in millions)

14. CONTINGENCIES AND COMMITMENTS (continued)

filed a notice of appeal and on June 4, 2024, the Ninth Circuit Court of Appeals reversed the district court's entry of final judgment and remanded the case to the district court for a new trial. The parties agreed to a settlement consisting of a cash payment of \$228.5, which was accrued in Other Accrued Expenses in 2024. On November 6, 2025, the court granted final approval of the parties' settlement. Sutter Health paid \$228.5 in 2025.

As of December 31, 2025, Sutter has approximately 64,000 employees. Approximately 27% of these 64,000 employees are represented by collective bargaining units, and 63% of the agreements have expired or will expire in 2026. Sutter is currently in negotiation or will be in negotiation during 2026 for successor agreements. While working to mitigate any potential work stoppages through successful negotiations, Sutter remains prepared to address the possibility of work stoppages. Should such disruptions occur, they may have a material adverse effect on the operations or on the financial position or results of operations of Sutter. Despite these uncertainties, Sutter Health remains committed to bargaining in good faith, evaluating positions that remain grounded in the realities of market pay and practices, reflecting business priorities, and acknowledging the needs of the workforce. Additionally, Sutter faces the difficulties of operating in a competitive and complex Northern California labor market.

Commitments: Sutter is currently compliant with pre-2030 seismic deadlines under a State of California law and continues investments to meet 2030 compliance. Sutter remains committed to prioritizing capital investments that strengthen access, affordability, and quality of care. Sutter evaluates the timing and scope of capital investments, facility utilization, and resource allocations based on its mission, federal and state regulations, and the rapidly evolving health care delivery environment. As of December 31, 2025, total current and board-approved commitments are estimated at \$6,200. This amount includes (i) approximately \$2,500 of legally binding commitments under executed contracts and (ii) approximately \$3,700 (unaudited) of board-approved capital plans that are not yet contractually obligated and may change as project scope, timing, and contracting decisions evolve.

15. SUBSEQUENT EVENTS

Sutter has evaluated subsequent events and disclosed all material events through March 4, 2026, which is the date these consolidated financial statements were issued.